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Name: اسماء صلاح عبدالجواد اسماعيل National ID: 27111210102688	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced MRI of the brain and orbits
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



نافع هرام



MDT FORM 345 / 8 / 20 25 G

Name	آلاء محمد عبد الجبار	MRN	4002-204
Age	53	ID	27121220202680
Menopausal status	Post	Phone	01225939052
NRS SCORE		Marital status	married
BMI		Family history	-ve
CO morbidity	-ve	REASON OF MDT	post op

Clinical Data

BX:	IDC GTL			R	L
ER:	8/8	PR:	8/8	HER 2:	0
				KI67:	15
Sonomammography:	Pre op (T2 - N1 - M0)				
Mets Workup:	Post (T2 N1)				
	TNM				
	Post NACT				

Recommendations: Post Rt skin sparing mastectomy and ALND (3/11)
T2 N1

MDT OUTCOME & PLAN

for AI + PORT + Zt 96ms

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name:	اسماء صلاح عبدالجواد	
MR. :	1560	
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
نتشرف بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انها		
The patient underwent a multidisciplinary team review in August 2025, during which an AI (adjuvant) endocrine therapy plan was initiated comprising PORTH, ZOETA, 6-month FEMARA 2.5 mg, CA (calcium) supplementation, and vitamin D for 3 months. On 11 April 2026, the medical oncology team documented that the patient had completed post-radiotherapy treatment for breast cancer with a CT3 N1 disease status, ER8/8, PR8/8, HER2-negative. The patient had received neoadjuvant chemotherapy (NACT) and was scheduled for RT SSM (surgical site management) on 26 July 2025. The plan included a YPT2 YPN1 schedule and a 3-month FEMARA course starting 9 January 2025. On 13 June 2026, the oncology team noted the initiation of a new schedule for ZOMETA and FEMARA, with a complaint of vertigo attributed to the current therapy. The plan was to assess the patient with a chest CT on 18 April 2024, which was noted as free of disease. On 11 July 2026, the attending physician recorded that the patient had a BIRADS 2 finding resolving, and the vertigo associated with FEMARA had improved after three months of therapy. The patient is currently on FEMARA for three months and is being monitored for symptom resolution.		
تحريرا في ,		
	الوقت :	التاريخ : 20-09-2026
	dina makram	الطبيب المعالج :
يعتمد ,		



S C A N

ID: 2854067

Date: 30/06/2025

Name: Asmaa Salah Abd Al Gawwad

Referred by: Prof.

Thanks for reference

Bilateral digital mammography , high resolution breast sonography & Elastography revealed :

Status follow up for a pathologically proven right breast cancer ; received 8 cycles of NAC; no available previous studies for comparison

- Bilateral scatted glandular condensations (ACR density B).
- An irregular shaped speculated margin dense mass is seen at the right UIQ with a metallic clip seen at its periphery .Targeted US revealed 1 o'clock, para-areolar palpable irregular shape speculated margin hypoechoic mass measuring 34x23x28 mm. Associated skin thickening and nipple retraction .
- Left UOQ dense lesion
- Normal skin thickness of the left breast .
- Multiple left scattered cysts the largest is at 3 o'clock = 20x25mm
- No left breast solid masses.
- Bilateral variable sized non-specific looking axillary lymph nodes.

OPINION :

Right breast UIQ locally advanced malignant mass under NCA , **BIRADS 6** as fore-detailed. .

Left breast cysts , **BIRADS 2**.

Kindly accept my best regards,

Dr.Mariam Raafat M.D.

Defining BIRADS classification of breast lesions according to the American Collage of Radiology:

Category	Suspected Cancer Risk
BIRADS 0 : Non-informative study.	
BIRADS 1 : Normal study.	0 %
BIRADS 2 : Benign findings.	0 %
BIRADS 3 : Probably benign.	2 %
BIRADS 4 : Probably malignant.	50 %
A : mild suspicion.	
B : moderate suspicion.	
C : high suspicion.	
BIRADS 5 : Malignant.	98-100%
BIRADS 6 : Pathologically proven malignant lesion.	

A MEMBER OF ALFA MEDICAL GROUP

الفا سكان

16171

Name: Mrs. Asmaa Salah Abdel-Gawwad Ismaeil

Date: 11/12/2024

ID:2224074390

Acc.:5058990

Referring / الجهاز المركزي للمحاسبات

Tc-99m MDP whole body bone scan

Technique:

- Whole body images were acquired 180 minutes after IV injection of ~15 mCi of the tracer.

Findings:

- Uniform tracer uptake all over the surveyed skeleton is noted with no evidence of any active spots.
- Degenerative changes of the spine are seen.
- Arthritic knee changes are noted.
- Both kidneys are visualized.

Opinion:

- No scintigraphic evidence of metastatic osseous deposits.*

Truly yours

Prof. Dr. Ahmed Kandeel. MD

Prof. of Nuclear Medicine, faculty of medicine, Cairo University

Printed by KH 14/12/24 9:04:49 AM

Patient Name : الأستاذ / اسماء صلاح عبد الجواد اسماعيل

Patient No. : 27111210102688

Path No. : 2362/25

Sex : Female

Age : 53 Y

Referring Doc :

Request No. : 13007726

Request Date : 26/07/2025

Verification Date : 30/07/2025

Financial Category : جوسفاروسي - مباره

Department : Out Patient

Registered by : marwa mohamed youss

Pathology Report

Clinical Data : Right breast cancer(Post NACT)

Nature of the specimen : Right skin sparing mastectomy &ALND

Gross Examination:

- Procedure: Right Skin sparing mastectomy specimen measured 14x14x4 cm , covering skin ellipse measured 8x2.5 cm .
- Tumor extent : An ill defined greyish white firm mass measured 2.3x2 cm .
- Distance from margins: 4 cm away from deep margin .
- Lymph nodes number and size: A fibrofatty tissue piece measured 5x3x2 cm enclosing 11 ALNs up to 1.5 cm .

Block designation :

1,2 mass

3.Mass +ink

4:7 Rubbery grey white area

8.Nipple

9.Deep

10 ,11,12 ALND

Grossing by :Dr.Heba El-shafeey

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against: GATA3 clone (L50-823).

Microscopic examination:

- Histological type: Invasive duct carcinoma , cellularity 80 %
- Grade (differentiation+ nuclear pleomorphism + mitosis): G 2(3+2+1)
- Angioinvasion : Positive
- Microcalcification : Positive
- Perineural invasion :Positive

Verified by :

Dr.Heba Hasan

Printing Date : 02/08/2025 12:15:18

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadenia - Cairo, Egypt

بجاء الطبيب

Patient Name : الأستاذ / أسماء صلاح عبد الجواد اسماعيل	Request No. : 13007726
Patient No. : 27111210102688	Request Date : 26/07/2025
Path No. : 2362/25	Verification Date : 30/07/2025
Sex : Female	Financial Category : جومات روسي - مبلره
Age : 53 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

- Ductal carcinoma in situ: Absent
 - Lymph node involvement: Positive (3/11)
 - Resection Margins: Free deep margin.
 - Additional pathologic findings: Positive microcalcification.
- IPT: GATA3:Positive

Diagnosis:

Right Skin sparing mastectomy & ALND, (Post NACT) :

Invasive duct carcinoma Grade 2 .

- Positive LNs (3/11) with extranodal extension. (showing therapy effect)
- Free nipple , free deep margin
- Positive microcalcification
- Stage : ypT2 ,ypN1
- RCB: 3.567 , RCB class: III , Tumor viability (80%). .

Dr. Walaa Ibrahim Ghoneim
Dr. Heba El-Shafeey

Verified by :

Dr. Heba Hasan

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Page 2 of 2

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Patient Name : الأستاذة / أسماء صلاح عبد الجواد اسماعيل
Patient No. : 27111210102688
Path No. : 4464/24
Sex : Female
Age : 52 Y
Referreing Doc :
Request No. : 13007825
Request Date : 18/11/2024
Verification Date : 25/11/2024
Financial Category : اورام دار السلام - مبانزه
Department : Out Patient
Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Right breast malignant looking solid mass.

Nature of Specimen: Tru-cut biopsy, collected at 17/11/2024.

Gross Examination:

Four grayish white tissue cores up to 1.7 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (2+2+2) Grade II.
- * Ductal carcinoma in situ: No.
- * Perineural invasion: No.
- * Angioinvasion: No.
- * Microcalcifications: Undetected.

Diagnosis:

Right breast malignant looking solid mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.
For further IHC (ER, PR, Her2, Ki67).

Signature

Dr. Salma Tarek.
Dr. Samia Ali.

Verified by :

Dr. Rofayda Abdallah

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Printing By : .marwa mohamed youss



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Asmaa Salah Abdel Gawwad Ismail	Date : 16/11/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 53 Year(s)
Medical ID: 27111210102688	Pat. ID: 94258

Combined bilateral breast mammography and complementary ultrasound

Indication:

Right breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast para-areolar s spiculated irregular mass lesion, of high density retracting the nipple.
- Complementary ultrasound revealed right breast irregular spiculated hypoechoic mass lesion, with subtle vascularity detected within on color Doppler, seen at 1 to 2 para-areolar position 1cm from the nipple involving the posterior aspect of the skin, measuring 2.6x3cm, consistent to the palpable lump.
- Bilateral fibrocystic changes of the breasts.
- Normal skin thickness, and nipple-areola complex of the left breast.
- Pathological right axillary lymph nodes level I (less than 3 in number) are seen with muffled hila, measuring 1.2X0.6 cm
- Nonspecific left axillary LN.

Opinion:

- Right breast para-areolar malignant looking mass lesion (BIRADS 5) for histopathological correlation.
- Pathological right axillary LNs.
- Bilateral fibrocystic changes of the breasts.(BIRADS 2)

Yours truly

Signature Dr. Youmna tarek

16/11/2024 1:12 PM